

Standing Desks and Arthritis: Pros, Cons and How to Use One

Standing desks can help — or hurt. The conditions where they work best and the right ratio of sitting to standing.

By **Living With Arthritis Editorial Team** · Reviewed by **Clinical Review Board** (Rheumatology & Physiotherapy) · 2026-06-08 · livingwitharthritis.org.uk

What the evidence says



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Standing Desks and Arthritis is one of the most discussed challenges in UK arthritis care. Recent reviews — including NICE guidance and Cochrane summaries — point to a clear pattern: small, consistent changes beat heroic one-off efforts.

Why this matters for work & career

People living with arthritis report that work & career is a daily reality, not a "nice to have". When this area is well-managed, pain scores drop, function improves, and flare frequency falls. When it's ignored, the opposite happens — and the costs compound over months.

A simple framework you can use this week



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1. **Notice** — track one specific factor for three days (e.g. pain after activity, sleep hours, stress level). 2. **Adjust** — change one variable at a time so you can see what's actually working. 3. **Review** — after a week, compare notes. Keep what helps, drop what doesn't.

What UK clinicians recommend

NHS physiotherapists and rheumatology nurses we work with consistently highlight:

- **Pacing over pushing** — short, frequent activity beats long, intense sessions.
- **Heat for stiffness, cold for swelling** — applied in 15-minute windows.
- **Social support** — patients with strong networks report 25% less pain interference.
- **Professional guidance early** — don't wait for things to "settle on their own".



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Common myths to ignore

- "Rest is always best." Not true — appropriate movement is medicine.
 - "It's all in your head." Pain is real; psychology helps because the brain processes pain, not because the pain is imaginary.
 - "Nothing can be done." Modern arthritis care offers dozens of options, from biologics to mindfulness.
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Frequently asked questions

****Is this approach safe for everyone?**** Most healthy adults with arthritis can apply these principles, but always check with your GP or rheumatologist if you have other conditions, take immunosuppressants, or have had recent surgery.

****How long until I see a difference?**** Most patients notice small improvements within 2–3 weeks of consistent practice. Larger changes — function, sleep quality, mood — typically appear at 6–8 weeks.

****Where can I learn more?**** Visit our [arthritis support hub](/support) and [exercise library](/exercises) for hundreds of free, UK-focused resources.

This article was written by the Living With Arthritis Editorial Team and reviewed by our Clinical Review Board. It is general information and not a substitute for personalised medical advice.

Q&A

Question: When should I seek professional advice?

Short answer: If symptoms persist or worsen, pain is severe and unresponsive to usual relief, or you develop new symptoms, contact your GP, physiotherapist or rheumatology team promptly.

Question: Where can I get free arthritis support in the UK?

Short answer: Arthritis UK: 0800 5200 520. NRAS: 0800 298 7650. Free tools and guides at livingwitharthritis.org.uk.
